

Meridian Health Group — Clinical Pathways & Escalation Policies

This document contains Meridian Health Group's operational clinical pathways for use in the MedAxis RAG guideline assistant. It covers key escalation policies, discharge planning guidance, and bed management protocols.

Section 1 — 30-Day Readmission Prevention Pathway

All patients identified as high risk for 30-day readmission (defined as predicted probability ≥ 0.35 from the MedAxis readmission model) must receive a structured discharge planning review before leaving the ward. This review must be completed by the responsible clinical team and documented in the EPR.

Required Actions — High Risk Patients

1. Confirm GP notification sent within 24 hours of admission
2. Community nursing referral submitted before discharge
3. Medication reconciliation completed and signed
4. Follow-up appointment booked (within 7 days of discharge)
5. Carer or next of kin contacted and discharge plan explained
6. Transport arranged if patient lacks capacity or mobility

Escalation Triggers

- NEWS2 score ≥ 5 at any point — escalate to registrar immediately
- O2 saturation $< 92\%$ on room air — escalate to respiratory team
- Creatinine rise $> 26 \mu\text{mol/L}$ in 48h — escalate to renal team
- Troponin $> 52 \text{ ng/L}$ — escalate to cardiology within 1 hour

Section 2 — Delayed Discharge Protocol

A delayed discharge is defined as a patient remaining in an acute bed beyond their Clinically Recommended Date of Discharge (CRDD). All delayed discharges must be coded with a delay reason category. The target is for fewer than 12% of discharges to be delayed (current rate: 22.7%).

Delay Reason Categories

Code	Category	Action Required
D1	Awaiting care package	Social worker must be notified on CRDD-3. Escalate if not resolved within 48h.
D2	Awaiting nursing home placement	Escalate to discharge team lead. Flag to bed manager daily.
D3	Patient/family decision	Document consent discussion. Involve PALS if delay > 5 days.
D4	Awaiting transport	Transport team SLA: 4 hours. Escalate if breached.
D5	Awaiting medication / TTO	Pharmacy SLA: 2 hours for TTO. Escalate if breached.
D6	Medical — awaiting test result	Specify test. Escalate to consultant if delay $> 24\text{h}$.
D7	Medical — patient not clinically ready	Must be reviewed by consultant daily.

Section 3 — Bed Management & Capacity Protocol

When bed occupancy exceeds 88%, the Bed Management Team activates the Escalation Framework. Escalation levels are:

Level	Trigger	Actions
Level 1	Occupancy 88–91%	Daily bed meeting moves to twice daily. Discharge lounge opened.
Level 2	Occupancy 91–95%	Emergency Department redirect considered. Elective admissions reviewed.
Level 3	Occupancy > 95%	OPEL 3 declared. Board-level notification. NHS England escalation if > 4 hours.
Winter Surge	Occupancy > 97% for >48h	OPEL 4. Chief Executive notified. Regional mutual aid requested.

Section 4 — AI Decision Support Policy

The MedAxis readmission risk model is classified as a decision support tool. It must not be used as the sole basis for any clinical decision. The predicted risk score should be used to prioritise clinical review, not to replace it. The following rules apply:

- A predicted risk score ≥ 0.35 triggers a mandatory structured discharge planning review (Section 1)
- A predicted risk score ≥ 0.65 requires a consultant-level review before discharge
- All predictions must be logged in the EPR with the model version used
- Clinicians may override the model at any time — all overrides must be documented
- The model must not be used for patients under 18, pregnant patients, or Day Case admissions

■■ REGULATORY NOTE: Use of this model in clinical pathways constitutes use of Software as a Medical Device (SaMD) under MHRA guidance. The model must be registered before go-live. Contact the MedAxis AI Governance team (Sophia Reeves) before any clinical deployment.

Section 5 — Medication Safety Guidance

High-risk medications require additional review at discharge. The following drug classes must have a medication reconciliation completed and documented before any patient is discharged:

- Anticoagulants (Warfarin, Apixaban, Rivaroxaban, Dabigatran)
- Insulin and hypoglycaemic agents
- Opioids and controlled drugs
- Immunosuppressants
- Lithium and antiepileptics (narrow therapeutic index)

Patients discharged on anticoagulants must receive written information on dose, indication, duration, and monitoring requirements. A copy must be sent to the GP on the day of discharge.